Why Enyrgy Exists, and What We Have Measured

Enyrgy Inc. Sunlight. Evolved.

We did not set out to build a wellness company. We set out to solve a problem that punched us in the face.

The moment

Late 2019. My wife Laura and I sat in a doctor's office looking at each other. Quiet. The kind of quiet where you are both trying to internalize something at the same time, and neither of you has the words yet.

Breast cancer.

I have spent thirty years building companies. Five exits. Over $500 million in enterprise value. I have been in hard rooms before, with hard numbers on the table, and I know how to work through a problem. This was not that kind of room.

The catalyst

A few months later, COVID hit. Laura is a front-line healthcare worker. Her immune system was not a wellness goal, it was a daily operational requirement. Protecting it became the conversation we had every morning and every night.

I had been working in the UV light space through my previous company, Anthem One, where we deployed UV-C sterilization technology adopted by Bell Helicopter and aerospace clients during COVID. We understood light-based biology at an engineering level.

We connected with physicians from LSU and Baylor who were running a clinical study using UVB light to support immune response in critically ill COVID patients. The results were a 60 percent reduction in mortality.

The number that changed everything

In those discussions, one of the doctors pulled up a piece of research I had not seen before. Women with a vitamin D level below 20 ng/mL have an over 80 percent higher risk of breast cancer.

Laura was at 17 ng/mL. Three points below the threshold.

I have been in enough data reviews to know what that number does and does not mean. It does not mean causality. It means the body we loved most was operating without something it needed, we had no way of knowing it, and no precise way of correcting it even if we had.

That number, 17, is the reason this platform exists.

The build

David and I looked at what existed in the market. Generic UVB lamps with no safety controls. Manual timers. No skin-type calibration. No dosing intelligence. No tracking. No way to know whether you were getting 500 IU of synthesis or 5,000.

It was not a product gap. It was a category gap. Precision phototherapy for the home did not exist.

We built it. Three years of engineering, then nine months of beta testing before it went to market.

The research foundation

We did not build a product and then find research to support it. We started with the research: four decades of published phototherapy science, and three of the most cited vitamin D researchers in the world.

The protocol behind every session was designed by **Dr. Samantha Kimball, Dr. Bruce Hollis and Dr. William Grant**, whose combined published work spans more than 500 peer-reviewed papers and 70,000 citations.

What four decades of science says

UVB-triggered vitamin D synthesis

- Holick, M.F. (2007). Vitamin D deficiency. *New England Journal of Medicine*, 357(3), 266 to 281.
- Webb, A.R., et al. (2017). Influence of season and latitude on the cutaneous synthesis of vitamin D3. *Journal of Clinical Endocrinology and Metabolism*.
- Chel, V., et al. (2008). Efficacy of different doses and time intervals of oral vitamin D supplementation. *Osteoporosis International*.

UVA-induced nitric oxide release

- Liu, D., et al. (2014). UVA irradiation of human skin vasodilates arterial vasculature and lowers blood pressure. *Journal of Investigative Dermatology*, 134(7), 1839 to 1846.
- Feelisch, M., et al. (2010). Is sunlight good for our heart? *European Heart Journal*, 31(9), 1041 to 1045.

UVA-triggered serotonin production

- Kaur, M., et al. (2019). Role of serotonin in seasonal affective disorder. *European Review for Medical and Pharmacological Sciences*.

Natural synthesis versus oral supplementation

- Heaney, R.P., et al. (2011). *Nutrients*, 6(10), 4472 to 4475.
- Luxwolda, M.F., et al. (2012). Traditionally living populations in East Africa have a mean serum 25-hydroxyvitamin D concentration of 115 nmol/l. *British Journal of Nutrition*, 108(9), 1557 to 1561.

This is independent literature. We cite it; we do not interpret it, and none of it is a study of the Enyrgy platform.

Third-party spectrum verification

An independent laboratory conducted spectral analysis of the device output and confirmed **90 to 95 percent UVB (280 to 315 nm) and 5 to 10 percent UVA.** That is the Triple-Pathway Advantage ratio referenced throughout the platform, verified by someone other than us. **The verification documentation is available on request.**

Sunlight, for comparison, is roughly 95 percent UVA. The device inverts the ratio deliberately, because vitamin D synthesis requires UVB in the 280 to 315 nm band, which visible-light and circadian lamps do not emit at all.

What we have measured

The 12-week study

Twelve weeks. Five healthy adults, skin types II to IV, averaging 5.1 sessions a week. Observational, no control group.

This is our own study, not independent research, and five people is a small study. We would rather publish every row than an average.

Participant	Skin type	Start, ng/mL	End, ng/mL	Change
P1	II	38.4	86.0	+124%
P2	III	34.0	88.4	+160%
P3	III	37.0	78.0	+111%
P4	IV	47.0	88.7	+89%
P5	III	43.4	79.9	+84%
Average		**39.96**	**84.20**	**+111%**

Every participant started insufficient or borderline, every one reached optimal, nobody dropped out, and there were zero adverse events. **The individual spread runs from +84 percent to +160 percent**, which is the more useful number: the average conceals how differently five people responded to the same protocol.

Study limitations, stated as we state them publicly

- Small sample size, N=5.
- No control group. This is observational data, not a randomized controlled trial.

- All participants were highly compliant, taking sessions in office. Real-world adherence will be lower.
- All participants were healthy adults without contraindications.
- The sample is not diverse enough to support population-level conclusions.

One further limit worth stating. Some people respond poorly to UVB because of polymorphisms in genes governing vitamin D metabolism, GC, CYP2R1 and DHCR7 among them. A user with these may not reach optimal, through no failure of the platform. That is a non-response caveat rather than a safety exclusion, and testing before and after settles it for any individual.

An independent trial is targeted for Q3 2027. The observational data is being compiled toward IRB and 510(k) pathway filings, where it will be tested properly rather than presented as more than it is.

What the 25,000 sessions do and do not tell you

Since that study, the platform has completed **more than 25,000 sessions with zero burns and zero adverse events**, across 600 plus users in five countries, with a return rate under 1 percent against an industry norm of 5 to 10 percent.

These two datasets answer different questions, and we keep them separate on purpose.

The 12-week study speaks to whether sessions move a vitamin D level. Five people, so it is suggestive rather than conclusive.

The 25,000 sessions speak to whether the dosing protocol is safe at scale. That is a much larger number and it is the one carrying real weight, but it is a safety record and not an efficacy result. It does not tell you what happened to anybody's blood work.

Conflating the two would let us present a five-person efficacy result as though 25,000 sessions supported it. They do not, and we will not.

Why the safety record holds

The design is the reason, not luck.

The spectrum is inverted. Sunlight is roughly 95 percent UVA. Enyrgy is over 90 percent UVB and under 10 percent UVA. Vitamin D synthesis requires UVB in the 280 to 315 nm band, which is what the platform is built to deliver and what visible-light and circadian lamps cannot produce at all.

The dose is calculated, not timed. The application reads each user's Fitzpatrick skin type, calculates that person's Minimum Erythemal Dose, and ends the session automatically. It also limits use to one session per day. Nobody estimates, and nobody watches a clock.

That is the difference between this and legacy tanning equipment, which delivers the wrong part of the spectrum through a manual timer with no dosing control.

The scientific advisory board

- **Dr. Bruce Hollis.** Over 70,000 citations and more than forty years on vitamin D research, much of it on the difference between oral supplementation and UVB-synthesized vitamin D. He has reviewed and endorsed the platform.
- **Dr. Samantha Kimball.** Designed the MED dosing protocol behind every session. Over 100 peer-reviewed papers and two decades in UV therapy safety.
- **Dr. William Grant.** NASA UVB researcher, with more than 350 peer-reviewed papers on vitamin D, sunlight and health outcomes.

What we believe, and how we say it

We are not a wellness brand. We do not sell transformation stories or lifestyle aesthetics. We sell a precisely engineered platform that gives the body a biological input it evolved to receive from sunlight, with precision and safety that sunlight never provided.

The published literature associates vitamin D status with immune function, and with a range of outcomes that people commonly manage by symptom rather than by cause. That literature also indicates that natural UVB synthesis produces a cascade oral supplementation does not replicate: vitamin D sulfate, regulatory photoproducts, nitric oxide release through a separate UVA pathway, and serotonin production.

We are careful about the line between those two paragraphs. The literature belongs to the researchers who produced it, and we cite it rather than interpret it. What belongs to us is the platform, the dosing protocol, and a number on a lab report that a user can watch move.

The honest case

The research behind Enyrgy is strong. Four decades of published science, three of the most credible researchers in the field, independent spectral verification, and cohort data showing every participant reaching optimal levels with zero adverse events.

We could present that without context. We chose not to.

The cohort study is small. The sample is not diverse enough to draw population-level conclusions. We are not a medical device, and we do not treat, diagnose, or prevent any medical condition.

What we are is a precision phototherapy platform grounded in peer-reviewed science, verified by independent spectral testing, and designed by researchers who have spent their careers studying exactly this mechanism.

That is the honest case. We are comfortable with it.

Enyrgy operates as a wellness device. It raises and documents vitamin D levels. Marketing claims are held to general wellness benefits, every clinical conclusion belongs to the user and their physician, the platform ships with a contraindication list, and the application will not run a session outside its calculated dose.

Scott Hansbury, Co-founder and CEO | enyrgy.com

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